Residential Treatment Facility
Inspection Report

Department of Health
P.O. Box 47874, Olympia, WA 98504-7874
TEL: 360-236-4732

September 20, 2021

Newport Academy - Seattle, 12029 113th Ave NE, Kirkland, WA. 98034-6901	Annie Gendaszek
Agency Name and Address	Administrator

NEW LICENSURE - OPERATIONAL	09/08/2021	Jennifer M Ross LMHC
Inspection Type	Inspection Onsite Dates	Inspector
X2021-254	RTF.FS.61015924	Co-occurring Services, ,
Inspection Number	License Number	RTF Services Types

Please note that the deficiencies/violations/observations noted in this report are not all-inclusive, but rather were deficiencies/violations/observations that were observed or discovered during the on-site inspection.

Deficiency Number and Rule Reference	Observation Findings	Plan of Correction
1385 Resident's individual service plan WAC 246-337-100(1)(a-b) (1) The licensee must develop and implement an individual service plan for each resident based on the resident's: (a) Initial health on admission; and (b) Health assessment(s).	The Washington Administrative Code STANDARD was NOT MET as evidenced by: Based on clinical record review and interview with staff, it was determined that the clinical record did not document an individual service plan that addressed the initial health on admission and the health assessment. Findings include: 1. In a review of clinical records, it was determined that 4 of the 4 files did not have an individual service plan that addressed the initial health on admission and the health assessment.	

Introduction
We require that you submit a plan of correction for each deficiency listed on the inspection report form. Your plan of correction must be submitted to the DOH within ten business days of receipt of the list of deficiencies.

You are required to respond to the Inspection Report with Noted Deficiencies by submitting a plan of correction (POC). Be sure to refer to the deficiency number. If you include exhibits, identify them and refer to them as such in your POC.

Descriptive Content
Your plan of correction must provide a step-by-step description of the methods to correct each deficient practice to prevent recurrence and provide information that ensures the intent of the regulation is met.

An acceptable plan of correction must contain the following elements:

- The plan of correcting the specific deficiency;
- The procedure for implementing the acceptable plan of correction for the specific deficiency cited;
- The monitoring procedure to ensure that the plan of correction is effective and that specific deficiency cited remains corrected and/or in compliance with the regulatory requirements;
- The title of the person responsible for implementing the acceptable plan of correction.

Simply stating that a deficiency has been "corrected" is not acceptable. If a deficiency has already been corrected, the plan of correction must include the following:

- How the deficiency was corrected,
- The completion date (date the correction was accomplished),
- How the plan of correction will prevent possible recurrence of the deficiency.

Completion Dates
The POC must include a completion date that is realistic and coinciding with the amount of time your facility will need to correct the deficiency. Direct care issues must be corrected immediately and monitored appropriately. Some deficiencies may require a staged plan to accomplish total correction. Deficiencies that require bids, remodeling, replacement of equipment, etc., may need more time to accomplish correction; the target completion date, however, should be within a reasonable and mutually agreeable time-frame.

Continued Monitoring
Each plan of correction must indicate the appropriate person, either by position or title, who will be responsible for monitoring the correction of the deficiency to prevent recurrence.

Checklist:

- Before submitting your plan of correction, please use the checklist below to prevent delays.
- Have you provided a plan of correction for each deficiency listed?
- Does each plan of correction show a completion date of when the deficiency will be corrected?
- Is each plan descriptive as to how the correction will be accomplished?
- Have you indicated what staff position will monitor the correction of each deficiency?
- If you included any attachments, have they been identified with the corresponding deficiency number or identified with the page number to which they are associated?

Your plan of correction will be returned to you for proper completion if not filled out according to these guidelines.

Note: Failure to submit an acceptable plan of correction may result in enforcement action.

Approval of POC

Your submitted POC will be reviewed for adequacy by DOH. If your POC does not adequately address the deficiencies in your inspection report you will be sent a letter detailing why your POC was not accepted.

Questions?

Please review the cited regulation first. If you need clarification, or have questions about deficiencies you must contact the inspector who conducted the onsite inspection, or you may contact the supervisor.

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EMPLOYEE AND CONTRACTOR SCREENING TOOL

- Has the individual had known contact with a confirmed or suspected case of COVID-19?
- Does the individual live with someone who is under investigation or awaiting testing for COVID-19?
- Is the individual experiencing any of the following symptoms?
    - Fever
    - Sore throat
    - Cough
    - Muscle pain
    - New loss of taste or smell
    - Shortness of breath
    - Nausea, vomiting, diarrhea

Vaccinated individuals do not need to quarantine following exposure to a confirmed case of COVID-19 as long as they remain asymptomatic since current exposure.

A. Excluding fully vaccinated people and those who have had COVID-19 in the past 3 months and recovered and do not develop new symptoms, the following individuals must quarantine following close contact and exposure to COVID-19:
    1. Any individual who lives with someone who is under investigation or awaiting testing for COVID-19 must remain at home until the test results come back negative. If test results come back positive, follow guidance under next point.
    2. Any individual who has been directly exposed to someone who has tested positive with COVID-19 within the past 14 days, with whom the employee has been living, been an intimate partner, or provided care, or with whom the employee has had prolonged contact (defined by the public health department as within 6 feet for 15 cumulative minutes or more over 24 hours), should quarantine and monitor symptoms daily. Quarantine can end after day 10 if there are no symptoms or after day 7 if the individual has a negative test between day 5 and day 7. The individual should self-monitor and practice masking through day 14. More details are outlined in Section VI.
    3. Employees who are showing symptoms of COVID-19 (e.g.: sore throat, cough, fever, chills, muscle pain, new loss of taste or smell, or shortness of breath) must refrain from coming to work until they have been symptom-free for a period of 24 hours (no fever without taking fever-reducing medicine) and have had at least 10 days since their symptoms appeared, whichever is later.
    4. Employees who have been diagnosed with COVID-19 must refrain from coming to work until they have been symptom-free for a period of 24 hours (no fever without taking fever-reducing medicine) and have had at least 10 days since their symptoms appeared, whichever is later.

The company will follow the guidance of the local health department and the CDC regarding notification, management, and reporting of positive employee cases.

NEWPORT HEALTHCARE COVID-19 MANAGEMENT GUIDELINES AND PROTOCOLS

ISSUED: 4/2/2020 REVISED: 4/7/2021

PURPOSE: The purpose of this guidance is to ensure Newport Healthcare employees are informed about COVID-19 and are prepared to respond as needed. This document coalesces previous distributed guidance and supersedes all previous distributions on the topic.

POLICY: Newport Healthcare follows the guidance of the Centers for Disease Control and Prevention (CDC), federal, state and local governments, related to prevention, treatment, and response to COVID-19. The following guidance, drafted by Newport Healthcare's COVID-19 Response Task Force in consultation with Dr. Rachel Fortune, is informed by CDC guidelines as well as federal, state, and local guidance.

During the COVID-19 outbreak, increased attention is being given to assessment, training, and management of symptomatic clients. As prevention is the key strategy to lessen the impact of COVID-19 broadly and on the organization, Newport Healthcare provides education to clients and employees, as well as supplies, to increase compliance with preventative measures. This document supplements the existing Infection Control policies and Policy and Procedure Manual and supersedes prior versions of the Respiratory Virus Protocol and supporting documents.

GUIDANCE: The following document provides guidance on

- Screening and assessment
- Management of symptomatic clients
- Personal Protective Equipment (PPE)
- Employee policies
- Communication, reporting, and notification
- Ongoing assessment and management
- On-site testing
- Clarification about exposure and return to work/programming guidance
- Vaccination

I. SCREENING

A. Clients are screened at time of Clinical Screening Questionnaire and again within 24 hours of scheduled arrival with the following questions:
   1. Has the potential client been vaccinated for COVID-19? If yes, provide verified COVID-19 vaccine card with dates and vaccine type.
   2. Has the potential client had a fever in the past 14 days (temperature 100.4 degrees or above)?
   3. Does the potential client currently have a fever? (100.4 degrees or above)
   4. Has the potential client or a close contact (someone with whom the potential client has been within 6 feet for 15 cumulative minutes or more over 24 hours) tested positive for COVID-19 (Coronavirus) or influenza within the past 14 days?

5. Has the potential client had contact with a confirmed case of COVID-19 in the community? If yes, refer to this CDC guidance for evaluation.
6. Is the potential client currently experiencing any of the following COVID-19 related symptoms?
    a. Fever
    b. Chills
    c. Muscle pain
    d. Cough
    e. Sore throat
    f. Shortness of breath or difficulty breathing
    g. New loss of taste or smell
    h. Nausea, vomiting, diarrhea

If YES to any of the above questions 2-6, client must receive clearance from the Medical Director. For cases that need clearance from an outside physician prior to admission, utilize *Medical Clearance Form for Admission to Behavioral Health Treatment Facility rev 3.31.2019*.

Clients arriving from congregate settings including psychiatric hospitals, other group homes, or correctional facilities may be required to provide proof of a negative PCR test within 72 hours of arrival to a Newport Healthcare facility.

Newport Healthcare may, under the guidance of the CDC and local health agencies, elect to require clients from areas with a high number of COVID-19 cases to undergo quarantine prior to admission to one of Newport's residential treatment programs.

Newport Healthcare's Admissions Department is provided with regularly updated guidance related to geographic areas or other conditions of admissions during the COVID-19 pandemic. This guidance is subject to change.

B. Onsite Screening at Residential and Outpatient Facilities: Upon arrival to a facility, clients are screened with the following procedure:
1. Temperature taken by staff
2. Is the potential client currently experiencing any of the following symptoms:
    a. Fever
    b. Chills
    c. Muscle pain
    d. Cough
    e. Sore throat
    f. Shortness of breath or difficulty breathing
    g. New loss of taste or smell
    h. Nausea, vomiting, diarrhea

Any client who answers YES to any symptom A through H should be discussed with the Medical Director at Residential or Director at Outpatient.

C. Onsite Testing: A facility may offer testing to clients as indicated for clinical assessment and screening. Each facility should follow their site-specific protocols for managing COVID-19. Any client pending test results should remain in isolation until results are received and course of action can be determined.

II. MANAGEMENT OF ILLNESS

A. Current Clients Presenting with COVID-19 Like Symptoms

If a client becomes symptomatic, measures are taken to evaluate, quarantine, and discharge when appropriate. All cases are evaluated individually and under the guidance of medical professionals. In general, sick clients should interact with only essential staff necessary for their safe and effective care during their time in isolation. Personal Protective Equipment (PPE) should be worn when a staff member is interacting with a client who is in quarantine for known or suspected COVID-19. When droplet precautions are in place, Droplet Precaution Signage must be posted.

B. Positive Client(s)

In the event of a positive test, the facility will contact Compliance, and Compliance will contact the local health department and follow their guidance on specific actions steps related to the situation.

Generally, best practice is for a COVID-positive resident to stay at the residential facility as long as symptoms remain mild and manageable at this level of care. Each facility has a site-specific protocol for managing illness and a sick client.

When possible, a quarantined client should be the only person with access to his/her bathroom and should receive all meals and medications in their space. Clients in isolation will use designated indoor and outdoor spaces separate from other clients to contain spread of the virus and utilize technology for virtual therapy to supplement programing. Staff bringing the clients meals and medications should wear CDC-approved PPE.

Discharge due to suspected or confirmed COVID-19 is determined by the Medical Director and logistical variables related to parent pick-up are taken into consideration. Parents are informed prior to admission of the possibility that their child may need to be picked up due to illness.

C. Each location has a site-specific protocol for managing a sick client. The Executive Director is responsible for oversight.

III. PERSONAL PROTECTIVE EQUIPMENT (PPE) & OTHER SAFETY MEASURES

A. Availability and Required Use
1. Every effort will be made to have PPE available at all facilities and the supply is to be distributed and monitored by the Executive Director or designee. The CDC is advising all facilities to implement strategies to conserve supplies during the COVID-19 pandemic.
2. During the COVID-19 pandemic, it is recommended that PPE is worn in the following circumstances:
    a. When droplet precautions are in place. Droplet Precaution Signage must also be posted.

b. When a staff member is interacting with a client who is in quarantine for known or suspected COVID-19.

3. PPE should also be worn in accordance with normal infection control plan.
4. PPE will be prioritized for healthcare workers who are coming into direct contact with individuals with known or suspected COVID-19.
5. Medical staff that are hands-on with patients and for medical reasons need to violate social distancing practices to do their jobs can be provided a mask daily.
6. Staff involved in the care of clients with known or suspected COVID-19 should adhere to the CDC's Standard, Contact, and Airborne precautions including eye protection, surgical masks, gowns, gloves, etc. These materials will be provided by Newport Academy to appropriate staff.
7. Clients with known or suspected COVID-19 should wear a surgical facemask when being evaluated medically and in the presence of others.
8. Items available: Gloves, masks, gowns, and goggles. N95 Respirators, which are needed for aerosol generating procedures that are not performed at Newport Healthcare, can be worn by staff but are not necessary in our environment.
9. Effective 2/2/2021, all staff providing direct care at sites with positive cases, confirmed Covid-19+ clients, and clients who are known to be exposed are encouraged to "double mask" and wear a disposable medical mask under a cloth mask.
10. Frequently Asked Questions about Personal Protective Equipment from CDC

B. Face Coverings

The CDC recommends that people wear masks in public settings, at events and gatherings, and anywhere around other people. Effective 2/2/2021, masks are required on planes, trains, buses, and other forms of public transportation in the United States.[1] Newport Healthcare staff wear face coverings while at work even when this is not mandated by a state or local mandate. Exceptions to this include when working alone in an office, taking a break for eating or drinking when alone, and performing duties outdoors when social distancing (maintaining 6 feet of distance between one another) is possible. Failure to comply with this guidance will result in appropriate action including verbal and written warnings.

C. Physical Distancing

All Newport staff and clients will strive to maintain 6 foot distancing whenever possible during the course of treatment and work day, especially during high risker periods such as meals when masks must be removed. In controlled environments such as the classroom, group room, and van transport, a 3 foot distance (shoulder to shoulder) is permitted to enhance access to treatment and academics during Covid-19 per CDC guidance[2] under the following conditions:

- All staff and clients ensure 100% mask compliance
- All chairs and seating is oriented to prevent direct face to face exposure
- Clients and staff are screened for symptoms each day prior to programming
- Structured or assigned seating to mitigate exposure risk
- Parental approval and advance screening measures required for van transport

[1] Your Guide to Masks, 2/2/2021 and Understanding the Difference Between Surgical Masks and Respirators

[2] https://www.cdc.gov/coronavirus/2019-ncov/community/schools-childcare/schools.html

IV. COMMUNICATION AND REPORTING POLICIES

The Newport Healthcare COVID-19 Task Force convenes daily to review protocols, plans, and communication. Determinations are made for how to effectively communicate internally and externally. Newport's Senior Director of Communications oversees outgoing messaging and communication. The Task Force works with local operators to review the effectiveness of this protocol and make adjustments as necessary. The Director of Compliance will log test results (negative or positive) from clients and the local HR representative will log test results for employees.

The Compliance department will oversee communication with all local health departments.

The Compliance Officer will monitor the CDC and other official sources regularly for updated guidelines and recommendations. The Leadership Team will discuss allocation of resources and finances to deal with prevention and response, messaging to stakeholders, and contingency plans.

V. EMPLOYEE EXPOSURE PROTOCOL

A. PURPOSE:
Our goal is to provide a safe work environment for our employees while providing continuing care for our clients for whom we are responsible. The purpose of this policy is to communicate the appropriate protocols so that we can implement them in a fair and consistent manner while working to keep everyone safe.

B. POLICY:
We follow current CDC guidance and considerations related to exposure to COVID-19.

1. Fully vaccinated individuals are not required to quarantine following exposure to COVID-19[3]. Vaccinated persons should continue to follow current guidance including wearing a face covering, avoiding crowds, practicing social distancing, and washing hands frequently.

2. Excluding fully vaccinated people and those who have had COVID-19 in the past 3 months and recovered and do not develop new symptoms, the following individuals must quarantine following close contact and exposure to COVID-19:
    - Any individual who lives with someone who is under investigation or awaiting testing for COVID-19 must remain at home until the test results come back negative. If test results come back positive, follow guidance under next point.
    - Any individual who has been directly exposed to someone who has tested positive with COVID-19 within the past 14 days, with whom the employee has been living, been an intimate partner, or provided care, or with whom the employee has had prolonged contact without both parties wearing a face covering (defined by the public health department as within 6 feet for 15 cumulative minutes or more over 24 hours), should quarantine and monitor symptoms daily. Quarantine can end after day 10 if there are no symptoms or after day 7 if the individual has a negative test between day 5 and day 7.

[3] https://www.cdc.gov/coronavirus/2019-ncov/vaccines/fully-vaccinated-guidance.html

The individual should self-monitor and practice masking through day 14. More details are outlined in Section VI.

- Employees who are showing symptoms of COVID-19 (e.g.: sore throat, cough, fever, chills, muscle pain, new loss of taste or smell, or shortness of breath) must refrain from coming to work until they have been symptom-free for a period of 24 hours (no fever without taking fever-reducing medicine) and have had at least 10 days since their symptoms appeared, whichever is later.
- Employees who have been diagnosed with COVID-19 must refrain from coming to work until they have been symptom-free for a period of 24 hours (no fever without taking fever-reducing medicine) and have had at least 10[4] days since their symptoms appeared, whichever is later.

The company will follow the guidance of the local health department and the CDC regarding notification, management, and reporting of positive employee cases.

When new guidance from the Center for Disease Control (CDC) is provided, we will adjust this policy as needed in alignment with this guidance. Should an issue arise within one of our programs, we will work directly with the local department of public health to best manage the situation. The Task Force that has been established within Newport is monitoring each program daily and will adjust program schedules as needed.

C. PROCEDURE:

Employees exposed to COVID-19 as defined above, or who have a temperature of 100.4 or higher, or who are showing any symptoms of COVID-19, should report to their manager or Human Resources that they will not be coming to work.

Your manager or someone from HR may inquire whether you are experiencing COVID-19 symptoms if you report feeling ill at work or if you call in sick. The information obtained will only be shared with HR but will allow us to know how to best protect you, our clients, and the workplace.

To reduce COVID-19 exposure and transmission, all employees are expected to abide by current COVID-19 safety precautions including wearing a face covering, frequent and thorough hand washing/sanitizing, and social distancing.

Time and Attendance

As an essential business, we have clients for whom we are responsible, so your attendance on site as an essential employee is critical during this time period in order for us to maintain a safe and healing environment for them.

Remote Work

The majority of the company's workforce is needed onsite to provide direct care to our clients and therefore the option to work remotely is limited for most. If a request to work from home is made, the position will be evaluated to determine if the individual can work remotely

[4] On August 10, 2020, CDC updated return to work guidance https://www.cdc.gov/coronavirus/2019-ncov/hcp/return-to-work.html; also see https://www.cdc.gov/coronavirus/2019-ncov/if-you-are-sick/quarantine.html (updated 12/2/2020)

(dependent upon the individual's ability to successfully perform their essential job duties on a remote basis, and the technical requirements associated with the role).

Travel
Non-essential business-related travel plans should be cancelled. All other travel should be reviewed with your supervisor to determine if it is mission critical.

D. EMPLOYEE SCREENING
Employee screening may be implemented at the facility level utilizing the plan identified at each facility. The Employee Screening Tool should be used (see page 9).

VI. EXPOSURE AND RETURN TO WORK OR PROGRAMMING GUIDANCE

A. See above Employee Exposure Protocol (V, B, 1) for criteria related to employee quarantine.
B. For individuals who do not meet the criteria listed above following exposure to COVID-19, the CDC approves the following alternatives to shorten the recommended 14-day quarantine[5]:
   1. 10 days - If no symptoms have been reported during daily monitoring, quarantine can end after 10 days
   2. 7 days – If no symptoms have been reported during daily monitoring, and a negative test is performed within 48 hours of the anticipated end of quarantine, then quarantine can end at 7 days. Quarantine cannot end sooner than 7 days.

In both scenarios, masking and symptom monitoring through day 14 are required. Should any symptoms appear at any point, the individual should self-isolate and notify their health care provider. Additional research appears here: Scientific Brief: Options to Reduce Quarantine

VII. ADDITIONAL RESOURCES

Interim Infection Control and Prevention Recommendations from CDC
Interim Guidance for Management of Influenza Outbreak in LTC Facilities
Strategies to Mitigate Staffing Shortages

Interim Public Health Recommendations for Fully Vaccinated People updated 3.8.2021

Updated Healthcare Infection Prevention and Control Recommendations in Response to COVID-19 Vaccination Updated 3.10.21

[5] Full text appears here: When to Quarantine Guidance

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Medical Clearance Form for Residential Admission to Behavioral Health Treatment Facility

Client Name: _____ DOB: _____

I confirm that I have evaluated the above client and attest to the following two statements:

Yes	No	
__	__	Does not require quarantine as part of disease management.
__	__	Is cleared for admission to a residential treatment facility where he/she will be living in close contact with others.

General comments:

_____ _____

Signature and Credentials Date

_____

Printed Name

_____

Phone Number

Wednesday, April 14, 2021

Crystal Goodwin, M.S.

Manager, Licensing and Compliance

Newport Academy | Newport Institute | Center for Families

Dear Ms. Goodwin:

Your agency Residential Treatment Facility and Behavioral Health Agency review is scheduled for Thursday, May 20, 2021. In an effort to ensure agencies are following social distancing guidelines and to meet your facilities needs during the COVID pandemic, we are asking that you provide our office additional information so we can best accommodate your facility during this remote review.

Since the COVID-19 pandemic may have temporarily changed schedules and suspended normal operations, please fill out the following questionnaire about scheduling your remote review:

☐ There is no change to our daily operation.

☐ Our days of operation have temporarily changed. During the pandemic we are operating on the following week days:

☐Monday ☐Tuesday ☐Wednesday ☐Thursday ☐Friday

☐ Our hours of operation have changed. During the pandemic our new hours are:

_____ to _____

☐ Our office is closed until _____

☐ What social distancing or other facility infection control requirements does your facility have in place? (Examples: masks or other PPE requirements, limiting the number of people in your facility, etc.) Enter response here

☐ Other comments or concerns? Enter response here

The Department of Health is committed to state social distancing measures to prevent the spread of COVID-19. We ask that we only have one facility point of contact, and that any entrance and exit conferences either be conducted electronically or that the facility's point of contact communicate results to the facility leadership.

Please email your response to your Reviewer by Friday, May 7, 2021 so we can determine when would be the best time to schedule your remote review.

STATE OF WASHINGTON
DEPARTMENT OF HEALTH
PO Box 47874 • Olympia, Washington 98504-7874

November 15, 2021

Annie Gendaszek
Washington Health Operations LLC
12029 113th Ave NE
Kirkland, WA 98034-6901

Subject: Inspection Number X2019-1112 and x2021-254

Dear Ms. Gendaszek,

The Washington State Department of Health conducted an inspection at Newport Academy on 9/8/2021 for both the RTF and BHA license. The Plan of Correction that was submitted was received on 11/8/2021 and approved on 11/15/2021. No further action is required.

I sincerely appreciate your cooperation and hard work during the inspection process and look forward to working with you again in the future.

Sincerely,

Jennifer Ross

Jennifer Ross, LMHC
Behavioral Health Reviewer